PEER REVIEW HISTORY

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ARTICLE DETAILS

TITLE (PROVISIONAL)	Comparisons of the Short-term Effectiveness and Safety of Surgical Treatment for Neovascular Glaucoma: A Systematic Review and Network Meta-Analysis
AUTHORS	Lin, Peijie; Zhao, Qian; He, Jing; Fan, Wei; He, Wenyi; Lai, Mingying

VERSION 1 – REVIEW

REVIEWER	El-Saied, Heba Cairo University
REVIEW RETURNED	09-Jun-2021

GENERAL COMMENTS	The paper is well written, addressing a challenging situation

REVIEWER	Park, Sang Chonnam National University Hospital
REVIEW RETURNED	22-Jul-2021

GENERAL COMMENTS	This is well-written paper. However, 12 months is a bit short to analyze the postoperative results of NVG. Even if the results are good up to 12 months, NV can recurs after that or the IOP rises. Therefore, how about changing the title to 'Comparison of the short-term effectiveness and safety...'. There are no special problems except for a few typos. Thank you.

REVIEWER	Murdoch, Ian UCL, Institute of Ophthalmology
REVIEW RETURNED	19-Aug-2021

GENERAL COMMENTS	The authors report a meta-analysis of papers reporting comparative studies of surgical interventions for neovascular glaucoma. This is essentially a repeat of a work reported by Dong et al. Whilst the authors point out some differences there are pros and cons to those and the work would benefit from some better critical appraisal of the difference in outcomes reported by the two studies which use much the same literature for their analyses. ‘our inclusion criteria were more rigorous, leading to more convincing results’ is insufficient debate. The project as presented is more a statistical exercise and less rooted in clinical understanding and appraisal of the papers (for selection bias etc) and why those outcomes might be seen I will give three examples:

	• A problem with neovascular glaucomas are that they have multiple causes and, in my view, the cause has a major effect on the outcome. For instance I have personally not really seen ocular ischaemic syndrome cases do well, diabetics can do well and retinal vein occlusions vary quite a lot in their behaviour, some doing brilliantly and others relentlessly slipping downhill. On this basis the mechanism of neovascularisation is, to my mind, a major potential confounding factor that has not been given any attention. • The therapy for neovascular glaucomas is two-fold. Firstly the underlying cause needs therapy which is what the anti VEGF and laser therapy is aimed at. Secondly the IOP needs lowering which is what the cyclo-ablation, trabeculectomy and tube surgery is aimed at. In my view the two need clarity of separation in order to interpret any findings. In addition IVAV only lasts for a limited period compared to laser therapy which is longer term. • The adverse events are inadequately analysed and discussed. These are critical to procedure acceptance and success. No denominators are given for the adverse effects and extracting these from elsewhere in the paper gives some possibly interesting observations. For example about a quarter of cases get hyphaema with drainage surgery in the absence of therapy for the neovascularisation (generally IVAV in most instances it seems). In those (tubes and trabeculectomies) with IVAV the proportion with hyphaema seems to be in the region of 10%. Other such observations are buried in the data and, in my view, worth exploring. A third issue I have is with the use of references. This is quite weak. The references quoted frequently do not specifically make the points the authors quote them as having made. This needs considerable attention to ensure the references say and prove what the text says they say! A fourth issue is a general problem I have with most meta-analyses. The fact is that a true meta-analysis needs the individual data from each constituent patient to make the statistics properly valid. This is, inevitably, a lot more effort to do but has significant rewards in terms of validity of any outcomes reported since several unquantifiable biases are removed. It is particularly relevant with such a heterogenous group of patients.

REVIEWER	Briz-Redón, Álavaro City Council of Valencia, Statistics Office
REVIEW RETURNED	01-Dec-2021

GENERAL COMMENTS	In this paper, the authors perform a meta-analysis to compare several treatments for neovascular glaucoma. In particular, a network meta-analysis is carried out for the comparison. The statistical methods followed by the authors are suitable and standard. I would only suggest the authors provide PrBest values as a probability (from 0 to 1), and not a percentage.

VERSION 1 – AUTHOR RESPONSE

Responses to the reviewers' comments:
Reviewer 1: Dr. Heba El-Saied, Cairo University
Comment: The paper is well written, addressing a challenging situation.
Response: We thank the reviewer for acknowledging the performance of this work and the quality of the presentation. We really appreciate the reviewer's positive feedbacks on our article.

Reviewer 2: Dr. Sang Park, Chonnam National University Hospital
Comment 1: This is well-written paper. However, 12 months is a bit short to analyze the postoperative results of NVG. Even if the results are good up to 12 months, NV can recurs after that or the IOP rises. Therefore, how about changing the title to 'Comparison of the short-term effectiveness and safety...'.
Response: We thank the reviewer for his or her valuable advice. According to the reviewer's suggestion, we have changed the title to "Comparison of the short-term effectiveness and safety...", see LOA 6.

Comment 2: There are no special problems except for a few typos.
Response: Thank you for your careful work. Based on the comment, we have removed all typos, see LOA 7.

Reviewer 3: Dr. Ian Murdoch, UCL
Comment 1: The authors report a meta-analysis of papers reporting comparative studies of surgical interventions for neovascular glaucoma. This is essentially a repeat of a work reported by Dong et al. Whilst the authors point out some differences there are pros and cons to those and the work would benefit from some better critical appraisal of the difference in outcomes reported by the two studies which use much the same literature for their analyses. 'our inclusion criteria were more rigorous, leading to more convincing results' is insufficient debate.
Response: Thank you for pointing this out. Several statements that we made were more ambiguous than intended, and we have adjusted to the text to be clearer.
There was insufficient evidence to determine the optimal surgical interventions in Dong's analysis listed as follows. First, intraocular pressure reduction and adverse events was not mentioned in the analysis. Second, follow-up time associated with success rate was unknown in 1/3 of the included studies,[1-9] and varied widely from 1 to 24 month in the others. [10-27] The significant heterogeneity indicated different degrees of bias in the analysis and may affect the assessment of surgical success. Third, we considered that retinal photocoagulation cannot be compared in Dong's analysis. There are reasons as follows: (1) 3 included studies did not mention whether retinal photocoagulation was done; [22, 24, 27] (2) it is a mistake in Dong's analysis to ignore the information that some patients underwent retinal photocoagulation in the included study; [21] (3) it is another mistake in Dong's analysis to extracted wrong data since patients actually had retinal cryocoagulation instead of retinal photocoagulation. [25] Fourth, iris photocoagulation and other glaucoma surgeries cannot be compared because 3 included studies did not mention whether other glaucoma surgeries were excluded. [13, 18, 21] Last, the result of Loop-specific approach showed a certain local inconsistency signifying that the findings related to trabeculectomy should be interpreted cautiously (P=0.0064). Therefore, we regrettably found that most of Dong's included studies were of poor quality and also failed to meet our inclusion criteria. Only two included studies in our meta-analysis were the

same as Dong's. [17, 19] The focus of our work is on more outcome measurements with 6 months and 12 months follow-up time, with stricter inclusion criteria and less bias, which is very different from Dong's work, and this is also the major contribution of our work. In addition, application of panretinal photocoagulation, laser peripheral iridotomy and anti-scarring drugs were common as adjunctive treatments, they could not be compared as stand-alone interventions. We have added the discussion on these issues in our revised manuscript, see LOA 8.

Included studies in Dong's research:

1 X. L, S. H. Therapeutic effect of modified trabeculectomy in treatment of neovascular glaucoma. *Int Eye Sci* 2014;14:745–6. [In Chinese].
2 S. C. Clinical effect observation of different treatments for neovascular glaucoma. *Pract Clin J Integr Tradit Chin West Med* 2015;15:69–70. [In Chinese].
3 J. X, Y. K, R. L, al e. EX-Press implantation combined with intravitreal injection of ranibizumab for neovascular glaucoma. *Recent Adv Ophthalmol* 2016;36:61–4. [In Chinese].
4 Y. B, J. L, L. T, al. e. Comparison of the effects of different surgical protocols on the levels of IL-6, PEDF and VEGF in serum and aqueous humor of patients with neovascular glaucoma. *Chin J Clin Rational Drug Use* 2015;8:161–2. [In Chinese].
5 Y. H, G. C, S. L. Clinical observation on the treatment of neovascular glaucoma by trabeculectomy combined with scleral buckling and ciliary body cryotherapy. *Jilin Med J* 2013;34:4220–1. [In Chinese].
6 N. G. Clinical application and observation of Ahmed glaucoma drainage valve implantation for neovascular glaucoma. *Chin Foreign Med Res* 2015;13:45–6. [In Chinese].
7 H. S. Clinical efficacy of Ahmed glaucoma drainage valve in the treatment of neovascular glaucoma. *J Today Health* 2016;15:91. [In Chinese].
8 J. S, X. L. The curative effect of Ahmed glaucoma valve combined MMC in neovascular glaucoma treatment. *Guide China Med* 2012;10:263–4. [In Chinese].
9 H. D. Clinical effect of trabeculectomy combined with mitomycin and interferon therapy on neovascular glaucoma. *Shenzhen J Integr Tradit Chin West Med* 2016;26:9–11. [In Chinese].
10 J. S, Q. W, Y. M, al e. The effect of Alpha Interferon in neovascular glaucoma filtration surgery. *Chin J Ophthalmol Otolaryngol* 1998;3:29–30. [In Chinese].
11 B. C, W. C, Y. H. Application of argon laser photocoagulation on iris surface new blood vessels in the surgical treatment of neovascular glaucoma. *Mod J Integr Tradit Chin West Med* 2005;14:316–7.
12 T. G, L. Z, H. W. Curative effect of laser coagulation of iris neovascularization combined with complex trabeculectomy for ueovas-cular glaucoma. *Chin J Ocular Trauma Occup Eye Dis* 2008;30:23–5. [In Chinese].
13 S. L, N. M, Shouyan. W, al e. Clinical analysis of intravitreal injection of Bevacizumab combined with panretinal photocoagulation and compound trabeculectomy for neovascular glaucoma. *Recent Adv Ophthalmol* 2010;30:1054–9. [In Chinese].
14 Z. L, P. Y. Combination treatment with cyclocryotherapy, trabeculectomy and mitomycin C for neovascular glaucoma. *Chin J Ocular Trauma Occup Eye Dis* 2010;32:837–8. [In Chinese].
15 H. Z, J. L, C. W, al. e. Panretinal photocoagulation and compound trabeculectomy for neovascular glaucoma. *World Health Digest* 2011;8:12–3. [In Chinese].
16 X. Z, Y. L, Z. L. Clinical observation of ciliary body cryotherapy, trabeculectomy and 5-FU in treatment of neovascular glaucoma. *Int Eye Sci* 2011;11:162-3. [In Chinese].
17 Mahdy RA, Nada WM, Fawzy KM, Alnashar HY, Almosalamy SM. Efficacy of intravitreal bevacizumab with panretinal photocoagulation followed by Ahmed valve implantation in neovascular glaucoma. *J Glaucoma* 2013;22(9):768-72.
18 M. S, X. L, C. Z, al. e. Observation of the effect of Ex-PRESS glaucoma drainage device implantation in the treatment of neovascular glaucoma. *China J Ctrl Endem Dis* 2014;29:245–6. [In Chinese].

19 Arcieri ES, Paula JS, Jorge R, Barella KA, Arcieri RS, Secches DJ, et al. Efficacy and safety of intravitreal bevacizumab in eyes with neovascular glaucoma undergoing Ahmed glaucoma valve implantation: 2-year follow-up. *Acta ophthalmologica* 2015;93(1):e1-6.
20 D. W, Q. Z. Clinical observation of trabeculectomy with mitomycin and interferon therapy on neovascular glaucoma. *Int Eye Sci* 2015;15:146–8. [In Chinese].
21 Y. Z, J. L, S. X. Therapeutic effects of Ex-PRESS glaucoma filtration device implantation in neovascular glaucoma. *International journal of ophthalmology* 2015;15:534–6. [In Chinese].
22 F. W, L. W. Safety of anti-VEGF drugs with trabeculectomy for neovascular glaucoma. *Int Eye Sci* 2016;16:837-40. [In Chinese].
23 J. C, P. S, X. G. Evaluation of subconjunctival injection of mitomycin C after Ahmed valve implantation treating neovascular glaucoma. *China Med* 2016;11:1400–3. [In Chinese].
24 Y. L, Q. M. Effective analysis of surgical treatment for neovascular glaucoma. *China Prac Med* 2016;11:95–6. [In Chinese].
25 D. K, Y. Z. Effects of intravitreal and subconjunctival injection of ranibizumab combined with trabecular filtering surgery in advanced stage neovascular glaucoma. *J Prevent Med Chin People's Liber Army* 2017;35:174–7. [In Chinese].
26 J. M. Trabeculectomy combined mitomycin and interferon in the treatment of neovascular glaucoma. *Jilin Med J* 2017;38:367–8. [In Chinese].
27 L. Z, Q. Y. Effect of surgical treatment on neovascular glaucoma. *Chin J Modrn Drug Appl* 2015;9:177–8. [In Chinese].

Comment 2: The project as presented is more a statistical exercise and less rooted in clinical understanding and appraisal of the papers (for selection bias etc) and why those outcomes might be seen I will give three examples:
Response: We thank the reviewer for his or her insightful comments and clear suggestions, which we believe improved the quality of our paper. As concerned, there are several problems that need to be addressed. According to the reviewer's profession suggestions, we have made extensive corrections to our previous draft. The detailed corrections are listed as follows.
Comment 2.1: A problem with neovascular glaucomas are that they have multiple causes and, in my view, the cause has a major effect on the outcome. For instance I have personally not really seen ocular ischaemic syndrome cases do well, diabetics can do well and retinal vein occlusions vary quite a lot in their behaviour, some doing brilliantly and others relentlessly slipping downhill. On this basis the mechanism of neovascularisation is, to my mind, a major potential confounding factor that has not been given any attention.
Response to comment 2.1: Thanks for the reviewer's reminding, which is of great importance to our article. As you are concerned, the primary ocular diseases leading to neovascular glaucoma have a major effect on the outcome. Based on the comment, we have read the articles again and extracted the data of composition of the primary diseases (Table 1). Most of the primary diseases were diabetic retinopathy and retinal vein occlusion, which may do better than ocular ischemic syndrome cases. To interpret the result cautiously, we have added a discussion on this potential confounding factor in our article, see LOA 9. We feel great thanks for your professional review work on our article.

Table 1. Composition of the primary diseases in the included studies

References	Study desig	Interventions	DR	RVO	OIS	CRAO	Others	Combine PRP
Ma,2012	OCT	AGV vs AGV+IVAV	26 vs 15	5 vs 5	1 vs 0	—	—	Yes

Mahdy,2013	RCT	AGV vs AGV+IVAV	15 vs 16	4 vs 4	1 vs 0	—	—	Yes
Zhou,2013	OCT	AGV vs AGV+IVAV	12 vs 7	10 vs 13	1 vs 2	2 vs 1	3 vs 2	Yes
Kang,2014	OCT	AGV vs AGV+IVAV	9 vs 12	2 vs 1	—	—	2 vs 1	Yes
Arcieri,2015	RCT	AGV vs AGV+IVAV	11 vs 10	9 vs 10	—	—	—	Yes
Kwon,2017	OCT	AGV vs AGV+IVAV	14 vs 33	3 vs 5	5 vs 7	—	—	Yes
Li,2019	OCT	AGV vs AGV+IVAV	14 vs 20	20 vs 26	—	—	—	N/A
Kong,2017	OCT	AGV vs AGV+IVAV vs AGV+IVAV	10 vs 12 vs1 2	10 vs 11 v s 7	1 vs 3 vs 2	—	—	N/A
Yildirim,2009	RCT	AGV vs CPC	NA	NA	NA	NA	NA	NA
Liu,2014	OCT	AGV vs CPC	13 vs 12	11 vs 9	—	—	7 vs 5	NA
Choy,2018	RCT	AGV vs CPC	6 vs 4	4 vs 1	1 vs 3	1 vs 0	—	Yes
Zhang,2014	OCT	AGV+IVAV vs Trab(MMC)+IVAV	5 vs 3	5 vs 5	—	—	3 vs 2	Yes
Sun,2017	OCT	AGV+IVAV vs Trab(MMC)+IVAV	—	—	—	—	—	Yes
Gao,2018	OCT	AGV+IVAV vs Trab(MMC)+IVAV	16 vs 16	16 vs 22	3 vs 1	1 vs 1	—	Yes
Du,2015	OCT	CCT vs Trab(MMC)+IVAV	19	17	—	—	10	Yes
Wang,2016	OCT	CCT vs Trab(MMC)+IVAV	14 vs 14	14 vs 13	—	—	4 vs 3	Yes
Saito,2010	OCT	Trab(MMC) vs Trab(MMC)+IVAV	22 vs 14	6 vs 6	4 vs 0	—	—	Yes
Takihara,2011	OCT	Trab(MMC) vs Trab(MMC)+IVAV	28 vs 18	4 vs 6	1 vs 0	—	—	Yes

			24		0			Yes
		Trab(MMC)	vs		vs			
Lee,2015	OCT	vs Trab(MMC)+IVAV	25	2 v 4	3	—	—	
			16					Yes
		Trab(MMC)	vs					
Yu,2018	OCT	vs Trab(MMC)+IVAV	18	26 vs 30	—	—	—	
			43					NA
		Trab(MMC)	vs					
Song,2019	OCT	vs Trab(MMC)+IVAV	43	39 vs 45	—	—	—	
			14		1			NA
			vs		vs	2 vs		
Shen,2011	OCT	AGV vs Trab(MMC)	13	4 vs 3	1	2	—	
			8		1			NA
		AGV	vs		vs			
Liu,2016	OCT	vs Trab(MMC)+IVAV	6	9 vs 10	0	—	—	

Notes: AGV, ahmed glaucoma valve implant surgery; CPC, cyclophotocoagulation; CCT, cyclocryotherapy; Trab, trabeculectomy; MMC, mitomycin; IVAV, intravitreal anti- vascular endothelial growth factor; DR, diabetic retinopathy; RVO, retinal vein occlusion; OIS, ocular ischemic syndrome; CRAO, central retinal artery occlusion; PRP, panretinal photocoagulation.

Comment 2.2: The therapy for neovascular glaucomas is two-fold. Firstly the underlying cause needs therapy which is what the anti VEGF and laser therapy is aimed at. Secondly the IOP needs lowering which is what the cyclo-ablation, trabeculectomy and tube surgery is aimed at. In my view the two need clarity of separation in order to interpret any findings. In addition IVAV only lasts for a limited period compared to laser therapy which is longer term.

Response to comment 2.2: Several statements that we made were more ambiguous than intended, and we have adjusted to the text to be clearer. We feel sorry that we did not provide enough discussion about the therapy for neovascular glaucoma (NVG). As you are concerned, the management of NVG includes both controlling of the underlying ischemic process and decreasing IOP. The first one is directed at reduction of ischemic drive that induces formation of new blood vessels, usually by panretinal photocoagulation or intravitreal anti-angiogenic administration. The second key aspect is the successful IOP management. This can be achieved with medical therapy and surgery, when ocular hypotensive drugs are insufficient. According to your suggestion, we have added a discussion to further explain the management of NVG in our revised manuscript, see LOA 10. Thanks again for pointing this out, which contributed a lot to improve the quality of our article.

Comment 2.3: The adverse events are inadequately analysed and discussed. These are critical to procedure acceptance and success. No denominators are given for the adverse effects and extracting these from elsewhere in the paper gives some possibly interesting observations. For example about a quarter of cases get hyphaema with drainage surgery in the absence of therapy for the neovascularisation (generally IVAV in most instances it seems). In those (tubes and trabeculectomies) with IVAV the proportion with hyphaema seems to be in the region of 10%. Other such observations are buried in the data and, in my view, worth exploring.

Response to comment 2.3: Thanks for the nice comment on our article. To make our results convincing, we have supplemented extra data of complications (Appendix 14) and summarized the adverse events (table 2). We had some interesting findings as follows: (1) complications including corneal edema, anterior segment inflammation, ocular pain, and ocular atrophy were reported more in CPC and CCT cases than others; (2) hyphema was reported more in AGV, CPC, CCT and Trab(MMC) cases, whereas was less in AGV+IVAV and Trab(MMC)+IVAV cases, which indicated that combined surgery with IVAV may decrease the incidence of hyphema; (3) low

IOP and shallow anterior seem to occur more in AGV+IVAV cases, may be due to the strong filterability of the implant at the beginning; (4) drainage tube related complications including tube exposure, tube occlusion and encapsulated plate occurred more often in AGV cases than those in AGV+IVAV cases, which demonstrated that anti-VEGF may improve surgical success rate of AGV; (5) bleb leak, vitreous hemorrhage and choroidal detachment were reported more in Trab(MMC) and Trab(MMC) +IVAV cases, and combined surgery with IVAV improved surgical effect; (6) it's worth noting that phthisis bulbi was reported in AGV and Trab(MMC) cases while retinal detachment was reported in AGV cases, although the incidence of these complications appeared low.

However, there were some differences in the adverse events included in each study, the results need to be interpreted with caution. According to the reviewer's thoughtful comment, we have added a discussion of adverse events in the revised manuscript, LOA 11.

Table 2. Adverse events

	AGV (n=213 eyes)	AGV+IVAV (n=269 eyes)	CPC (n=33 eyes)	CCT (n=53 eyes)	Trab(MMC) (n=182 eyes)	Trab(MMC)+IVAV (n=278 eyes)
Corneal edema	20 (9.4%)	5 (1.9%)	14 (42.4%)	31 (58.5%)	3 (1.6%)	—
Hyphema	70 (32.9%)	38 (14.1%)	8 (24.2%)	13 (24.5%)	68 (37.4%)	28 (10.1%)
Low IOP	16 (7.5%)	24 (8.9%)	3 (9.1%)	—	7 (3.8%)	17 (6.1%)
Shallow anterior	20 (9.4%)	40 (14.9%)	—	—	10 (5.5%)	11 (4.0%)
Vitreous hemorrhage	6 (2.8%)	1 (0.4%)	—	—	11 (6.0%)	6 (2.2%)
Choroidal detachment	12 (5.6%)	10 (3.7%)	—	—	18 (9.9%)	22 (7.9%)
Retinal detachment	2 (0.9%)	—	—	—	—	—
Tube exposure	12 (5.6%)	12 (4.5%)	—	—	—	—
Tube occlusion	13 (6.1%)	5 (1.9%)	—	—	—	—
Encapsulated plate	19 (8.9%)	12 (4.5%)	—	—	—	—
Phthisis bulbi	6 (2.8%)	—	—	—	2 (1.1%)	—
Bleb leak	—	2 (0.7%)	—	—	6 (3.3%)	6 (2.2%)
Anterior segment inflammation	19 (8.9%)	20 (7.4%)	11 (33.3%)	16 (30.2%)	—	1 (0.4%)
Ocular pain	8 (3.8%)	2 (0.7%)	18 (54.5%)	36 (67.9%)	5 (2.7%)	2 (0.7%)
Ocular atrophy	—	—	2 (6.1%)	—	1 (0.5%)	—

Notes: AGV, ahmed glaucoma valve implant surgery; CPC, cyclophotocoagulation; CCT, cyclocryotherapy; MMC, mitomycin; Trab(MMC), trabeculectomy with MMC; VEGF, vascular endothelial growth factor; IVAV, intravitreal anti-VEGF; AGV+IVAV, AGV combined with IVAV; Trab(MMC)+IVAV, Trab(MMC) combined with IVAV

Comment 3: A third issue I have is with the use of references. This is quite weak. The references quoted frequently do not specifically make the points the authors quote them as having made. This needs considerable attention to ensure the references say and prove what the text says they say!
Response: Thanks for the reviewer's careful checks. We are sorry for our improper quotation of the references. Based on the comment, we have made the corrections to make the references harmonized with viewpoints in the manuscript, see LOA 12.

Comment 4: A fourth issue is a general problem I have with most meta-analyses. The fact is that a true meta-analysis needs the individual data from each constituent patient to make the statistics properly valid. This is, inevitably, a lot more effort to do but has significant rewards in terms of validity of any outcomes reported since several unquantifiable biases are removed. It is particularly relevant with such a heterogenous group of patients.
Response: This is a valid question, and we are actively pursuing the answer from different research institutions. However, we feel sorry for not receiving the individual data after sending letters of request to the research institutions. In consideration of this problem, we have set a relatively strict selection criteria for inclusion to strengthen the reliability of the result. Thank you for highlighting the importance of this issue.

Reviewer 4: Dr. Álavaro Briz-Redón, City Council of Valencia
Comment: In this paper, the authors perform a meta-analysis to compare several treatments for neovascular glaucoma. In particular, a network meta-analysis is carried out for the comparison. The statistical methods followed by the authors are suitable and standard. I would only suggest the authors provide PrBest values as a probability (from 0 to 1), and not a percentage.
Response: We thank the reviewer for his or her suggestion. We have revised it as a probability (from 0 to 1) instead of a percentage, which make the table more understandable, see LOA 13.

According to the associate editor and reviewers' comments, we have revised the manuscript extensively. If there are any other modification we could make, we appreciate to modify them earnestly and hope that the correction will meet with approval.
Thanks again for the editor and reviewers' review work and we hope to learn more from you.
Yours Sincerely,
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VERSION 2 – REVIEW

REVIEWER	Murdoch, Ian UCL, Institute of Ophthalmology
REVIEW RETURNED	11-Feb-2022

GENERAL COMMENTS	Many thanks you have addressed all of my main concerns. I wonder if a short sentence on adverse effects might usefully be added to the abstract?

REVIEWER	Briz-Redón, Álavaro City Council of Valencia, Statistics Office
REVIEW RETURNED	10-Feb-2022

GENERAL COMMENTS	I have no further comments.